Nab-paclitaxel · every 3 weeks

NCCP 00230 · Source reconciled; independent clinical, pharmacy and patient review pending

PRACTICAL TREATMENT GUIDE

Your infusion and access plan

Nab-paclitaxel on Day 1 every 21 days. Review response after two cycles; the NCCP protocol says to discontinue if there is no response. Further treatment depends on benefit and tolerance.

STEP 1

Before the first visit

Confirm the indication, baseline bloods, required cardiac checks, medicines review and consent before the first infusion.

STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Nab-paclitaxel is infused over 30 minutes. Blood review, preparation and any observation add to the total visit time. Staff observe for reactions. Report burning, pain, swelling or leakage at the infusion site immediately.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit length

Nab-paclitaxel is infused over 30 minutes. Blood review, preparation and any observation add to the total visit time. A port is optional access, not a requirement for a take-home pump. This regimen has no routine ambulatory pump.

Premedication

Routine allergy premedicines are usually not required for this albumin-bound formulation. Low-risk antiemetic support is separate; previous reactions may change the plan.

Sources: NCCP 00230 v7 · official source · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient review pending.